

WHEN SOMETHING HAS JUST HAPPENED TO YOUR PARENT

THE FIRST 72 HOURS

What to do first — before the system starts making decisions for you.



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From **It's Not Your Fault: Helping Caregivers with Aging Loved Ones** · TheCardinalsPromise.com

Nobody picks the day they become a caregiver. The phone rings, or a doctor says a word you weren't ready for, or a discharge planner asks where your parent is going next — as if you're supposed to already know. You're not. You are an ordinary person being handed an extraordinary job: no training, no map, no time. These pages are the first piece of the map — what to do in the first three days, the questions that protect you, and the numbers to call. You don't have to know everything today. You just have to know the next right step.

EMERGENCY

If someone is in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This guide helps you plan. It does not replace emergency care.

In the First 24 Hours — Do These

SEVEN ACTIONS, NOT SEVEN CHAPTERS

1. **Handle immediate safety** — the person, the stove, the car keys, the stairs.
2. **Photograph the insurance and Medicare cards**, front and back.
3. **Gather the medication list** — or the bottles, in one bag (Chapter 25).
4. **If a hospital is involved, ask:** "Is my parent admitted as an inpatient, or under observation?" Write down the answer and who gave it — Chapter 13 explains why this one question matters so much.
5. **Find out who is legally authorized to make decisions**, and where the power of attorney papers are (Chapters 19–20).
6. **Start a notes page** — what changed, when it started, who told you what.
7. **Then turn the page and find your situation.** That is your next right step.

In the next 24–72 hours: read the chapter that matches your situation, hold a short family meeting (Chapter 5), check the caregiver's own limits honestly (Chapter 4), and start the Important Documents Folder (Chapter 19) and the Emergency Contacts and Crisis Plan (Chapter 26).

ROB'S NOTE

Families usually know more than they think.

They may not know the diagnosis. They may not know the benefit rules. But they know when something feels different.

I have watched families talk themselves out of that feeling because they did not want to upset anyone. I understand that. No one wants to be the person who says, "Mom is not safe anymore."

But noticing a change is not betrayal. It is care. You are not taking over. You are paying attention. And sometimes, paying attention is the first loving thing you can do. It can turn panic into a plan.

If This Is Happening, Start Here

Find the line that sounds like your family. That is your starting point — and the chapter listed is where the full guide takes you deeper.

IF THIS IS HAPPENING	IN THE FULL GUIDE
Mom fell, seems weaker, or is suddenly less steady	Chapter 2: Falls, Weakness, and Decline
Dad is forgetting medications, bills, appointments, or directions	Chapter 3: Memory Loss and Dementia
Your parent is repeating questions or acting confused in new ways	Chapter 3: Memory Loss and Dementia
The caregiver is exhausted, angry, resentful, or overwhelmed	Chapter 4: Caregiver Burnout
One person is doing everything and the family is not helping	Chapter 4 and Chapter 5: The Family Meeting
The family is arguing, avoiding decisions, or blaming each other	Chapter 5: The Family Meeting
Your parent is coming home from the hospital and you feel unprepared	Chapter 6: Home Health — or Chapter 10 if a rehab facility comes first
Your parent needs help bathing, dressing, cooking, or staying safe	Chapter 7: Private Duty Care
Living alone does not seem safe anymore	Chapter 8: Assisted Living
Doctors are talking about comfort, symptoms, or quality of life	Chapter 11: Palliative Care
You are wondering if it may be time for hospice	Chapter 12: Hospice
Death may be getting close	Part 5: Hospice and the Last Days
You are wondering whether Medicare pays for assisted living	Chapter 8: Assisted Living and Chapter 13: Medicare Basics
You are trying to understand home health versus hospice	Chapter 6: Home Health and Chapter 12: Hospice
You need a sitter or daily help and are not sure that is "home health"	Chapter 7: Private Duty Care (Chapter 6 explains the difference)
Prescriptions are getting too expensive	Chapter 17: Prescription Assistance

You are confused about Medicare, Medicaid, insurance, or who pays	Part 3: Paying for Care
You do not know where the legal, medical, or financial papers are	Part 4: The Documents That Matter

The Question That Protects Your Wallet

\$ THIS COULD SAVE YOU THOUSANDS

If your parent is in a hospital bed tonight, ask one question tomorrow morning, and ask it exactly like this: **“Is my parent admitted as an inpatient, or under observation?”** The bed looks the same either way — but observation status can quietly change what Medicare pays toward rehab afterward. Ask, write down the answer and who gave it, and if the answer is “observation,” ask the doctor whether inpatient admission is appropriate. The full guide walks through this trap — and the rest of the paying-for-care maze — step by step.

WHAT NOBODY TELLS YOU

Discharge moves fast — often faster than families expect, and sometimes on a Friday afternoon. You are allowed to say: *“We do not yet have a safe plan at home. What are our options?”* Saying it calmly, early, and to the discharge planner by name is often the difference between a safe transition and a preventable readmission.

WHO TO CALL

- **Any aging question, any state** — Eldercare Locator, 1-800-677-1116
- **Medicare questions** — your state SHIP office (free counseling)
- **Medicaid** — your county Department of Social Services; an elder law attorney for planning
- **Veteran or surviving spouse** — a Veterans Service Officer
- **Memory concerns / wandering** — Alzheimer’s Helpline, 1-800-272-3900 (24/7)
- **Caregiver in crisis** — call or text **988**, any hour
- **Finding senior living** — a local placement advisor, or **Cardinal Care Bridge** at TheCardinalsPromise.com — no cost to you

Two Sheets to Fill In Tonight

These two pages are yours to write on. The medication list is the single most useful document a family can hand a doctor, a hospital, or a pharmacist — make it tonight, photograph it, and keep a copy with you.

■ WORKSHEET — MEDICATION AND DOCTOR LIST

Create one clear list for appointments, emergencies, hospital visits, home health, hospice, assisted living, memory care, or rehab.

Person receiving care _____

Date last updated _____

Medication-list person / phone _____

Primary pharmacy / phone _____

Allergies & medication reactions _____

Current medication list

MEDICATION	DOSE	HOW OFTEN / TIME	REASON	PRESCRIBER

As-needed medications

MEDICATION	DOSE	WHEN USED	MAX ALLOWED

Over-the-counter, vitamins & supplements

ITEM	DOSE	HOW OFTEN	REASON

Recently stopped medications

MEDICATION	DATE STOPPED	WHO STOPPED IT?	REASON

Medication concerns (check any that apply)

- | | |
|---|--|
| ☐ Missed or extra doses | ☐ Dizziness, falls, or sleepiness |
| ☐ Confusing instructions | ☐ New confusion or poor appetite |
| ☐ Duplicate medications | ☐ Nausea or constipation |
| ☐ Old bottles still in use | ☐ Trouble swallowing pills |
| ☐ Multiple pharmacies or prescribers | ☐ Refusing medications |
| ☐ High cost | |

Doctor & care team list

NAME	ROLE / SPECIALTY	PHONE	NEXT VISIT
Primary doctor			
Specialist			
Specialist			
Pharmacy			
Home health / hospice			
Facility nurse			

Medication safety plan

Who fills the pill box, and how often? _____

Who checks for missed doses? _____

Where are medications stored? _____

Are old medications removed from the home? (yes / no / needs attention)

Who updates the list after changes? _____

■ WORKSHEET — EMERGENCY CONTACTS & CRISIS PLAN

Create one clear emergency page. The goal: the right person can find the right number fast.

Person receiving care / date of birth _____

Address _____

Main diagnosis / conditions _____

Allergies _____

Medication list location _____

Preferred hospital _____

Important documents folder location _____

Family point people

ROLE	NAME	PHONE
Main family point person		
Backup point person		
Healthcare agent		
Main caregiver		
Backup caregiver		

Medical contacts

CONTACT	NAME / ORGANIZATION	PHONE	AFTER-HOURS
Primary doctor			
Specialist			
Pharmacy			
Home health			
Hospice			
Facility nurse			

Call 911 for (check to confirm the caregiver knows)

- | | |
|---|---|
| ☐ Chest pain | ☐ Unconsciousness |
| ☐ Severe trouble breathing | ☐ Uncontrolled bleeding |
| ☐ Stroke signs — sudden face drooping, arm weakness, or speech trouble (note the time) | ☐ Serious fall or major injury |
| | ☐ Severe allergic reaction |

☐ Fire or unsafe environment

☐ Immediate danger to self or others

Hospice / home health

On hospice? Yes ☐ No ☐ Receiving home health? Yes ☐ No ☐

Hospice agency / main & after-hours number _____

Hospice nurse _____

Home health agency / number _____

Instructions from the team _____

Documents & medical orders (check what applies)

☐ Healthcare POA

☐ Medication & allergy list

☐ Living Will / Advance Directive

☐ Insurance / Medicare / Medicaid / VA cards

☐ DNR / MOST / POLST

☐ Long-term care insurance

Family call chain

ORDER	WHO GETS CALLED?	PHONE	WHO CALLS THEM?
1			
2			
3			

Before you call — what changed?

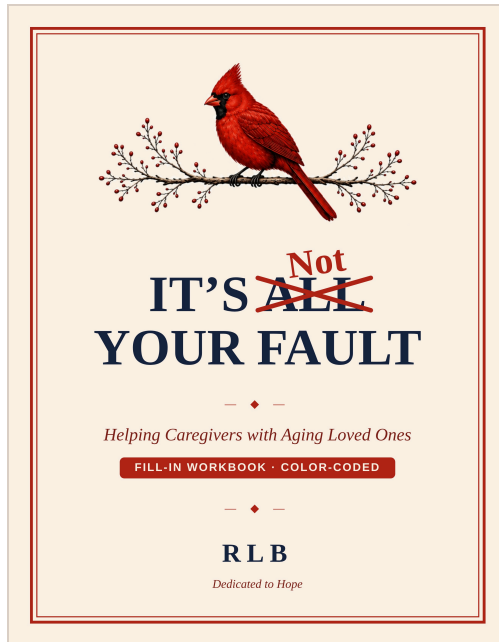
Awake & responsive? Yes ☐ No ☐ Unsure ☐ Fall? Y ☐ N ☐ Breathing trouble? Y ☐ N ☐

Chest pain? Y ☐ N ☐ New confusion? Y ☐ N ☐ Fever? Y ☐ N ☐ New pain? Y ☐ N ☐

Medication change? Y ☐ N ☐

Someone Had to Pay Attention. It Turned Out to Be You.

Nobody applauds this work. There is no job title, no training day, no pay — just an ordinary person at a kitchen table at 2 a.m., trying to learn Medicare, medicine, and the truth about their own family all at once. You are not failing at this. You were never given the map. **These pages were the first piece of it. The book is the rest.**



It's Not Your Fault: Helping Caregivers with Aging Loved Ones picks up where this handout stops: every care option compared honestly, how families actually pay (Medicare, Medicaid, VA, long-term care insurance), the documents that protect the people you love, word-for-word scripts for the hardest conversations, and hospice explained by someone who has spent a career beside it — with a complete fill-in workbook bound into the back.

Color-coded by urgency — **RED: Act Today** · **PURPLE: Watch Closely** · **BLUE: Plan Ahead** — so you can find the right page on the worst day.

Get the book today — on Amazon, or at TheCardinalsPromise.com. The worst day is easier with the map already on the shelf.



KEEP GOING

Free printable worksheets, current program links & caregiver resources

TheCardinalsPromise.com/resources

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